

PLABABLE

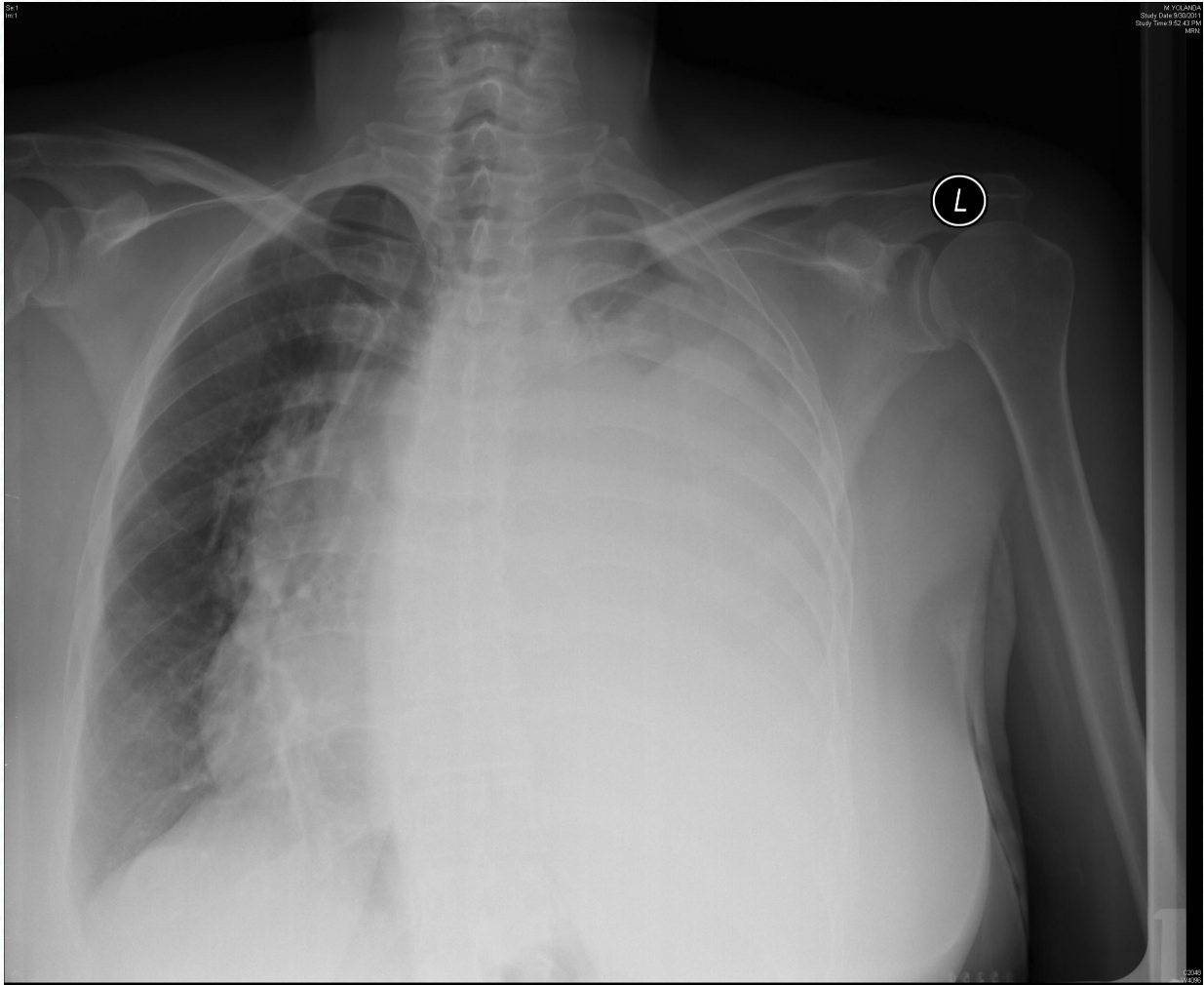
GEMS 

VERSION 1.1

PALLIATIVE CARE



Pleural Effusion



Features:

- Worsening cough
- Shortness of breath
- Pleuritic chest pain

Management:

!! Pleural Aspiration !!

Mental Capacity

1. Does the patient understand the information relevant to decision?
2. Does the patient able to retain the information?
3. Can the patient use and weigh up the information as part of the process in making the decision?

Some patient can lack of capacity to make some decision but have capacity to make others

Mental capacity can fluctuate with time

In palliative care patients:

- If they develop a diseases or complications e.g. pneumonia
- Discuss their wish on management plan if **they have mental capacity**

Pain Ladder

Increase dose,
replace with
stronger option
or add on

One step at a time
❌ Do not change to
analgesic on the same step
❌ Do not jump
e.g. codeine to oxycodone

Strong opiates
e.g. morphine, fentanyl*,
BEFORE oxycodone,
diamorphine

Weak opiates
e.g. codeine,
dihydrocodeine,
tramadol*

Simple analgesia
e.g. paracetamol or
NSAID (including
topical)

NEVER GO DOWN

- *Fentanyl patches has slow onset **NOT** suitable for acute pain*
- *If patient develops side effects from oral morphine e.g. nausea, vomiting or drowsiness*
- *→ Switch to oral oxycodone*

Oxycodone has **DOUBLE POTENCY** of
morphine but fewer side effects

Pain Ladder

Brain trainer:

A patient is taking morphine but is still in pain and experiencing side-effects. What is the most appropriate step in management?

➔ **Replace morphine with oxycodone**

Pain Ladder

Brain trainer:

A palliative care patient is using fentanyl patches but still has pain. She has difficulty swallowing. What is the most appropriate step in management?

→ **Administer morphine sulphate subcutaneous in a syringe driver and keep using fentanyl patches**

Pain Ladder

Brain trainer:

A cancer patient has a liver metastasis and RUQ pain. He takes paracetamol and morphine sulfate regularly. Aside from increasing his dose of morphine sulphate, what other medications could be added to help with his pain?

→ **Ibuprofen or naproxen**

This is not moving down on the ladder because you will continue giving morphine sulfate

Severe Pain Control In Palliative Care

Severe pain in terminal cancer

Pain relief medication

End of life care

Refer to pain ladder

Refer to hospice

- A place for patient with terminal illness
- A place to help alleviate pain and suffering
- A place to provide emotional, psychological and social support

Euthanasia (painless killing of patient with terminal diseases) is a **CRIME** in UK!!!!

Primary → Secondary Care

Brain trainer:

A palliative care patient in the GP office is suicidal.
What is the most appropriate action?

→ Refer to hospice care

Oral Intake

Brain trainer:

A patient taking morphine has constipation. What is the most appropriate management?

→ **Senna**

Morphine

Brain trainer:

A terminal elderly woman is unable to eat, has not opened her bowels for a week, lacks capacity, is not lucid and has been bedridden for a month. Her daughter expresses concern about her oral intake. What is the most appropriate action?

➔ **Encourage small sips of water and mouth care**

Codeine Phosphate

- ✓ Comes in Tablets
- ✓ Comes in solution
- ✓ Comes in injection (intramuscular)
- ✗ Does not come in subcutaneous injection

Has high side effects profile:

- Nausea & vomiting
- Constipation
- Confusion

DO NOT give to elderly due to side effects

Alternative



options

Buprenorphine patches
OR
Subcutaneous morphine

DO NOT use oral route if patient is
nauseated

Codeine Phosphate

Brain trainer:

An elderly man with pain which is well controlled on the maximum dose of oral codeine has swallowing difficulties. What is the most appropriate action?

→ **Switch oral codeine phosphate to subcutaneous morphine**

Death Rattle

Brain trainer:

What is an appropriate treatment for excessive respiratory secretions in palliative care?

- **Glycopyrronium bromide subcutaneously**
- or
- **Hyoscine butylbromide subcutaneously**

Persistent Cough

Brain trainer:

A patient's pain is well controlled using codeine linctus but he is complaining of a persistent dry cough. What is the next most appropriate management?

→ Oral morphine

Opiates act centrally to inhibit the cough reflex

Capsular Pain Management

Examples:

Patient with liver cancer can experience 'liver pain' (right hypochondriac pain)

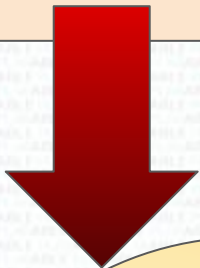
Already on morphine and paracetamol, what is the next step?



Answer:

Liver has no nerves, pain is due to liver enlargement (stretching of liver)

Pain relief = NSAID



Capsular pain responds well to NSAID

✗ Do not add codeine ✗

We do not go backwards on the pain ladder, patient is already on morphine

Medications For Nausea And Vomiting (L-CHOP)

Levomepromazine

N&V associated with Parkinson's' diseases
NEVER use Haloperidol

Cyclizine

1.Increased ICP
2.Hyperemesis gravidarum (1st line)

Haloperidol

1.Renal Failure
2.Hypercalcaemia
3.Drug or Toxin induced

Ondansetron

1.Chemotherapy
2.Radiotherapy
3.Post-operative N&V

Buccal
Prochlorperazine

1.Vertigo
2.Meniere's
3.BPPV
4.Vestibular Neuritis

Haloperidol side effects: sexual dysfunction, gynaecomastia, neuroleptic malignant syndrome

Parkinson's Disease

Brain trainer:

A man with Parkinson's disease is complaining of vomiting and nausea. What antiemetic would benefit him?

→ **Levomomepromazine**

Nausea & Vomiting

Causes:

Glioblastoma (intracranial tumour)

→ ↑ Intracranial pressure

Causing 🤢 nausea and vomiting 🤮

Treatment:

First-line antiemetic for nausea & vomiting secondary to ↑ intracranial pressure is

Cyclizine

Dexamethasone

1. Shrink oedema around tumour
2. Reduce intracranial pressure
3. Relieve nausea & vomiting

Hypercalcaemia ($\text{Ca}^{2+} > 2.6$) In Palliative Care

Features:

Neurological: Lethargy, confusion, depression

Gastrointestinal: Constipation, nausea, vomiting

Renal: Polyuria ($\uparrow$ Urination),
polydipsia ($\uparrow$ Thirst)

Cardiovascular: ECG changes
 $\rightarrow$ Short QT interval

Treatment:

IV Fluid



IV bisphosphonate
e.g. Alendronate or Zoledronate

Remember:

DNR = Do not resuscitate
= Do not perform cardiopulmonary resuscitation

DOES NOT MEAN DO NOT GIVE TREATMENT

Bleeding In Palliative Care

**Severe bleeding
e.g. haematemesis**



**Administer
subcutaneous
midazolam or
morphine**

Remember

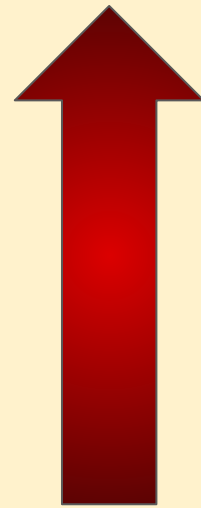
**Aim of hospice is to facilitate a
more comfortable last hours of
life while dying**

Bone Metastasis

*Diagnosed by **MRI** then **bone scintigraphy***

Most common origin area (s)

1. Spine
2. Pelvis
3. Ribs
4. Skull
5. Long bones



Most Common

Females



Males



Breast

Prostate

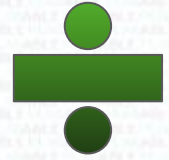
Lung

Analgesic Calculation

Main Dose
(twice daily)



Sum of all
morphine to
achieve pain
relief

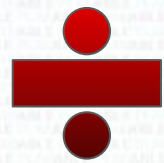


2

Breakthrough
dose
(every 4
hourly prn)



Sum of all
morphine to
achieve pain
relief



6

OR

Sum of all
morphine to
achieve pain
relief



10%

Up to 6 times daily
when required

Example:

Patient's pain is controlled by morphine 20 mg four times daily, what is her main and breakthrough dose?

Answer:

Main dose = $(20\text{mg} \times 4) / 2 = 40 \text{ mg}$ twice daily

Breakthrough dose

= $(20\text{mg} \times 4) / 6 = 13 \text{ mg}$ every 4 hourly **or**

= $(20\text{mg} \times 4) \times 10\% = 8 \text{ mg}$ every 4 hourly

Writing A Death Certificate

Fourth of
July **NOT**
04/07

Date of death
in **words** ✓

Small cell
carcinoma of
main right
bronchus
NOT lung
cancer

Clearly ✓

Specifically ✓

Diseases or condition
directly leading to death

Vague terms ✗
Eg.respiratory
diseases

Modes of
dying ✗
Eg.cardiac
arrest

Abbreviation ✗

Anticipatory Medications

These are medications that are allowed to be given to patient during the last days of life

Only given **SUBCUTANEOUSLY**

- **SC Morphine**
→ Pain and breathlessness
- **SC Haloperidol**
→ Nausea and vomiting
- **SC Midazolam**
→ Anxiety, delirium, agitation
- **SC Hyoscine butylbromide**
→ Noisy respiratory secretion

Controlled Drug Prescription Requirement

Pharmacy Stamp		Age		Title, Forename, Surname & Address	
		D.o.B 27/4/1942		John Hayes 33 Faye Street Bart's Square SE1 9FB	
Please don't stamp over age box				NHS Number: 7083332569	
Number of days' treatment N.B. Ensure dose is stated					
Endorsements					
Zomorph 10 mg modified-release capsules					
Supply 56 capsules MISTAKE HERE					
Take one capsule twice a day					
[No more items on this prescription]					
Signature of Prescriber					
Date					
3/5/2019					
For dispenser No. of Prescns. on form					
Anypractice Dr. A. Robinson 6509111 Jane Road London SE1 9BD Tel: 0011 122 234					

Writing both age and date of birth is good practice.
NOT a legal requirement unless < 12 years old

Control drugs quantity must be in both words and figures
Example: Supply 56 (fifty-six) capsules

Valid for 28 days only

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